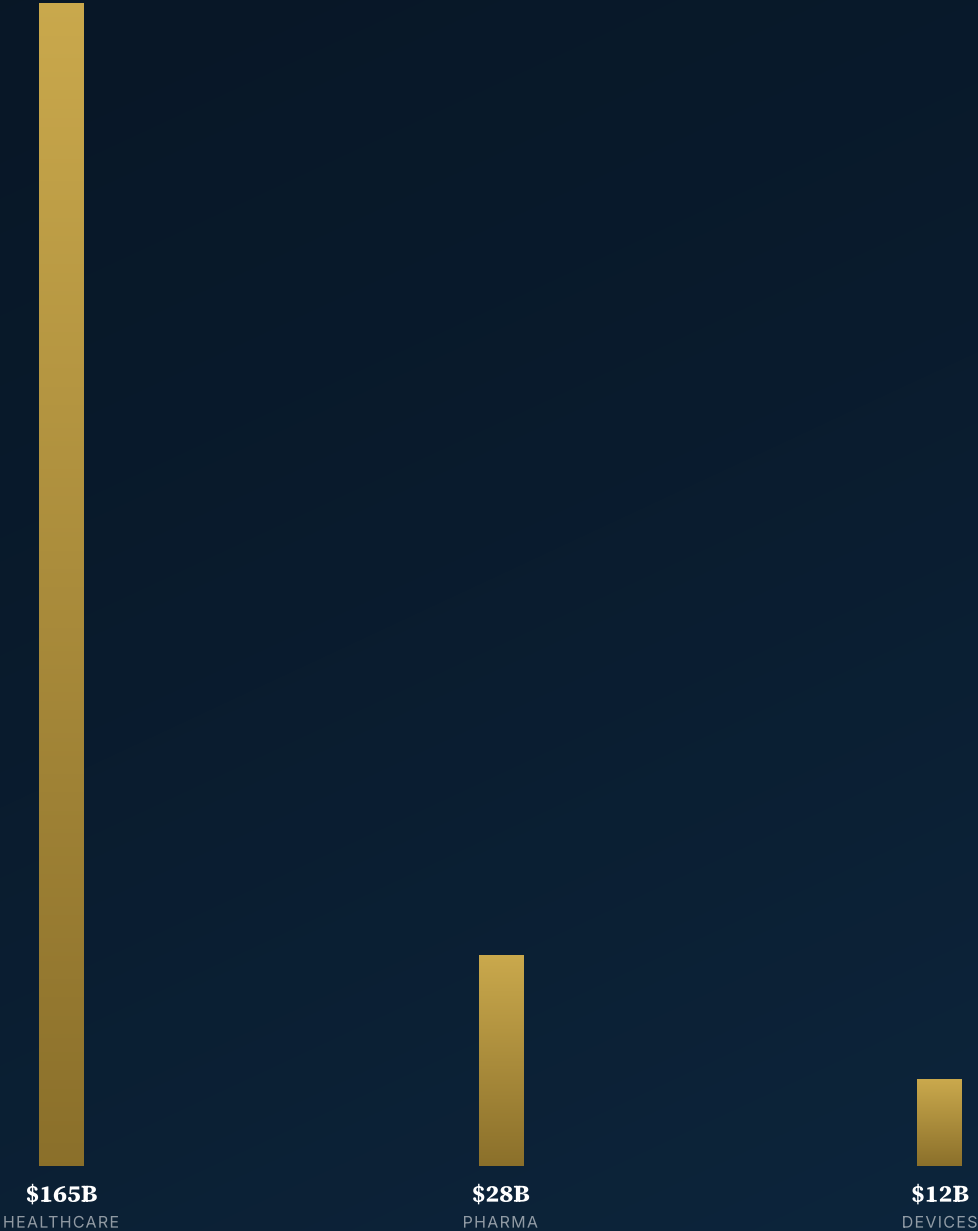


PUBLISHED 2026 CHIPS

Headline midpoints from bionixus.com/brazil-healthcare-market-report. Dual channel: SUS volume + ANS premium.



SAMPLE EDITION · SEPTEMBER 2026

Brazil Healthcare Market Report 2026

ANVISA, SUS, CONITEC and ANS intelligence for Latin America's largest healthcare market — dual-channel public procurement and premium private hospital access.

COVERAGE	FOCUS	USE
Brazil	Pharma · MedTech · Access	SUS & ANS planning

01 Contents

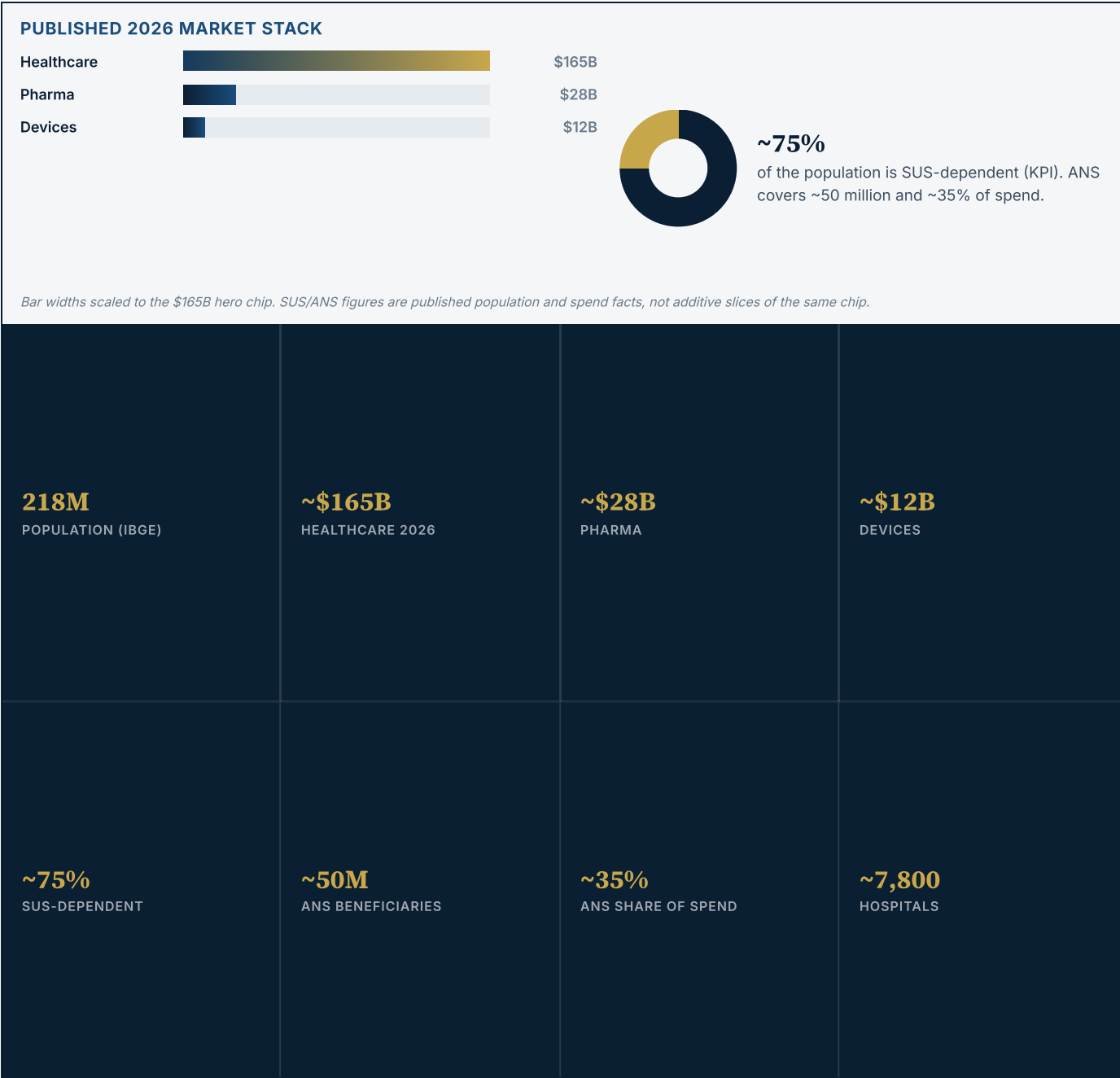
Brazil at a glance	03	Pharmaceutical market	04
Medical devices market	05	Therapy and epidemiology	06
ANVISA, CONITEC and ANS clock	07	Hospitals and dual-channel accounts	08
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This PDF is a **gated sample** aligned to **bionixus.com/brazil-healthcare-market-report** (published 27 May 2026; modified 22 August 2026) and the Brazil market-intelligence KPIs on that page. Not a syndicated SKU database. Not legal, medical or investment advice.

WHAT THIS SAMPLE IS — AND IS NOT

It is a board briefing from BioNixus published intelligence: sizing bands, regulators, HTA steps and named private hospitals. Therapy-area dollar sizes are not invented — the Brazil KPI panel does not publish therapy \$ bands, so therapy is taken from the page FAQ in qualitative form only.

Live page: [bionixus.com/brazil-healthcare-market-report](#) · Related: [/usa-healthcare-market-report](#) · [/canada-healthcare-market-report](#) · [/news/bionixus-brazil-office-latam-expansion-2026](#)



02 Brazil at a glance

218M POPULATION 2026 (IBGE)	~\$165B HEALTHCARE MARKET 2026	~\$28B PHARMA MARKET 2026	~\$12B DEVICES MARKET 2026
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Brazil is Latin America's largest healthcare market and a dual-channel commercial system: **SUS** (Sistema Único de Saúde) — constitutionally guaranteed public care — alongside **ANS**-regulated supplementary private insurance covering about **50 million** people and roughly **35%** of total healthcare spend.

PUBLISHED SERIES	HEALTHCARE	PHARMA	DEVICES
Page hero / FAQ (headline)	USD 155–175B · ~\$165B	USD 26–30B · ~\$28B	~\$12B
On-page market-intelligence KPIs	USD 165–185B · 9.9% of GDP	USD 26–30B (INTERFARMA / BioNixus)	USD 11–13B (ABIMO)

CAPACITY

Population 218 million (IBGE). GDP per capita USD 11,500 (IMF 2025). ~7,800 hospitals (public SUS ~5,900; private supplementar ~1,900). ~500,000 beds (2.3 per 1,000). FAQ also states 215+ million citizens under the SUS constitutional guarantee.

INSTITUTIONS

ANVISA (Classes I–IV). CMED (PCFAR / PMC price caps). CONITEC (SUS HTA). ANS (Rol de Procedimentos). 220+ domestic pharmaceutical manufacturers. ~75% of the population is SUS-dependent (KPI).

Hero chips and FAQ on /brazil-healthcare-market-report. KPI panel: BioNixus Brazil market intelligence.

HOSPITAL CENSUS (PUBLISHED)

7,800 Hospitals	5,900 Public SUS	1,900 Private	500k Beds
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Hospital counts scaled to 7,800 = 100%. The beds bar is a separate series (500,000) and is not on the same unit scale — label only.

218M POPULATION (IBGE)	~\$165B HEALTHCARE 2026	~\$28B PHARMA	~\$12B DEVICES
~75% SUS-DEPENDENT	~50M ANS BENEFICIARIES	~35% ANS SHARE OF SPEND	~7,800 HOSPITALS

03 Brazil pharmaceutical market

Published size: **USD 26–30 billion in 2026** (FAQ and KPI; hero midpoint **~\$28B**) — the largest pharmaceutical market in Latin America.

ANVISA REGISTRATION

Required before any product can be marketed. Priority track PRE (Pedido de Análise Prioritária) for unmet need. Standard review **12–36 months** by complexity (FAQ). KPI clock: priority 120 days; regular / new biological 365 days; biosimilar comparative 120 days. Module 1 in Portuguese. WHO PQ or EMA/FDA reference expedites.

CMED PRICE REGULATION

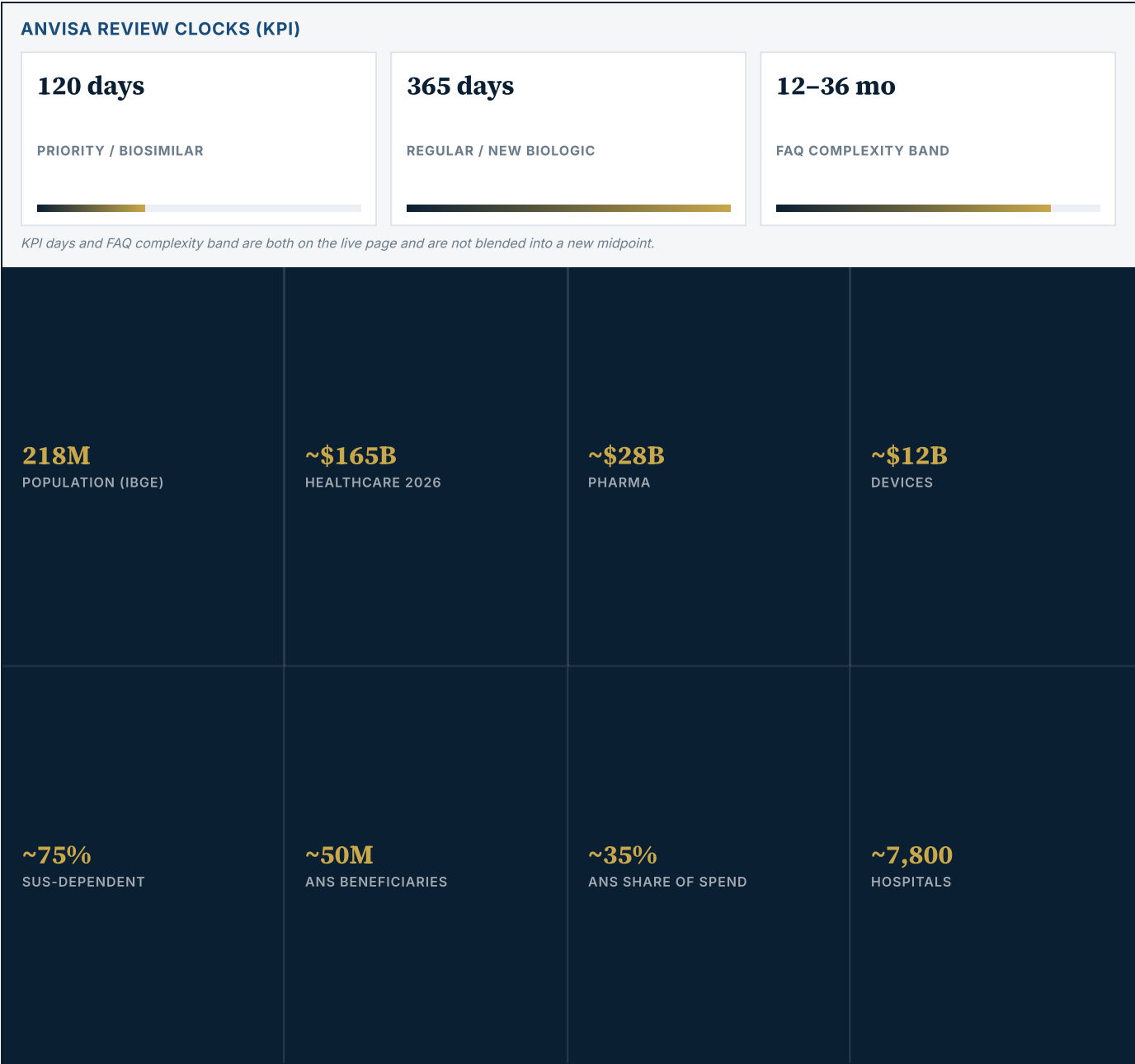
CMED (Câmara de Regulação do Mercado de Medicamentos) sets maximum prices using PCFAR (Preço Fábrica) and PMC (Preço Máximo ao Consumidor). International reference basket includes the US, Canada, Australia and EU5.

LARGEST THERAPY AREAS (FAQ — QUALITATIVE, NO INVENTED \$)

Oncology — rapidly growing; SUS expansion via RENAME and INCA-affiliated hospital protocols. **Diabetes** — world’s sixth-largest diabetic population; insulin, GLP-1, SGLT-2. **Cardiovascular** — high-volume antihypertensives, statins, anticoagulants; generic-dominant. **Infectious diseases** — HIV/AIDS fully funded by SUS; globally recognised ARV access programme. **Immunology and biologics** — TNF, IL-17, JAK growing in the private supplementary market; biosimilar adoption accelerating.

CONITEC incorporation — not ANVISA approval — is the gate to SUS volume. Private ANS accounts can generate revenue earlier, at international price points with lighter price regulation than SUS.

FAQ and executive summary on /brazil-healthcare-market-report. ANVISA timeline detail also in the Brazil KPI registration path.



04 Brazil medical devices market

~\$12B HERO CHIP 2026	\$11–13B KPI PANEL (ABIMO)	30–60% EFFECTIVE IMPORT TAX
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Hero chip ~\$12B; KPI USD 11–13 billion (ABIMO). Import taxes at a 30–60% effective rate are a published market-access barrier; Drawback Suspension is used by importers.

DEVICE / PRODUCT ACCESS (COMPRESSED FROM THE PUBLISHED PATH)

- 1

ANVISA classificação — Classes I (low risk) to IV (high risk).
- 2

Registration — GAFAR/GGMED; CNPJ required; approval valid 5 years, renewable.
- 3

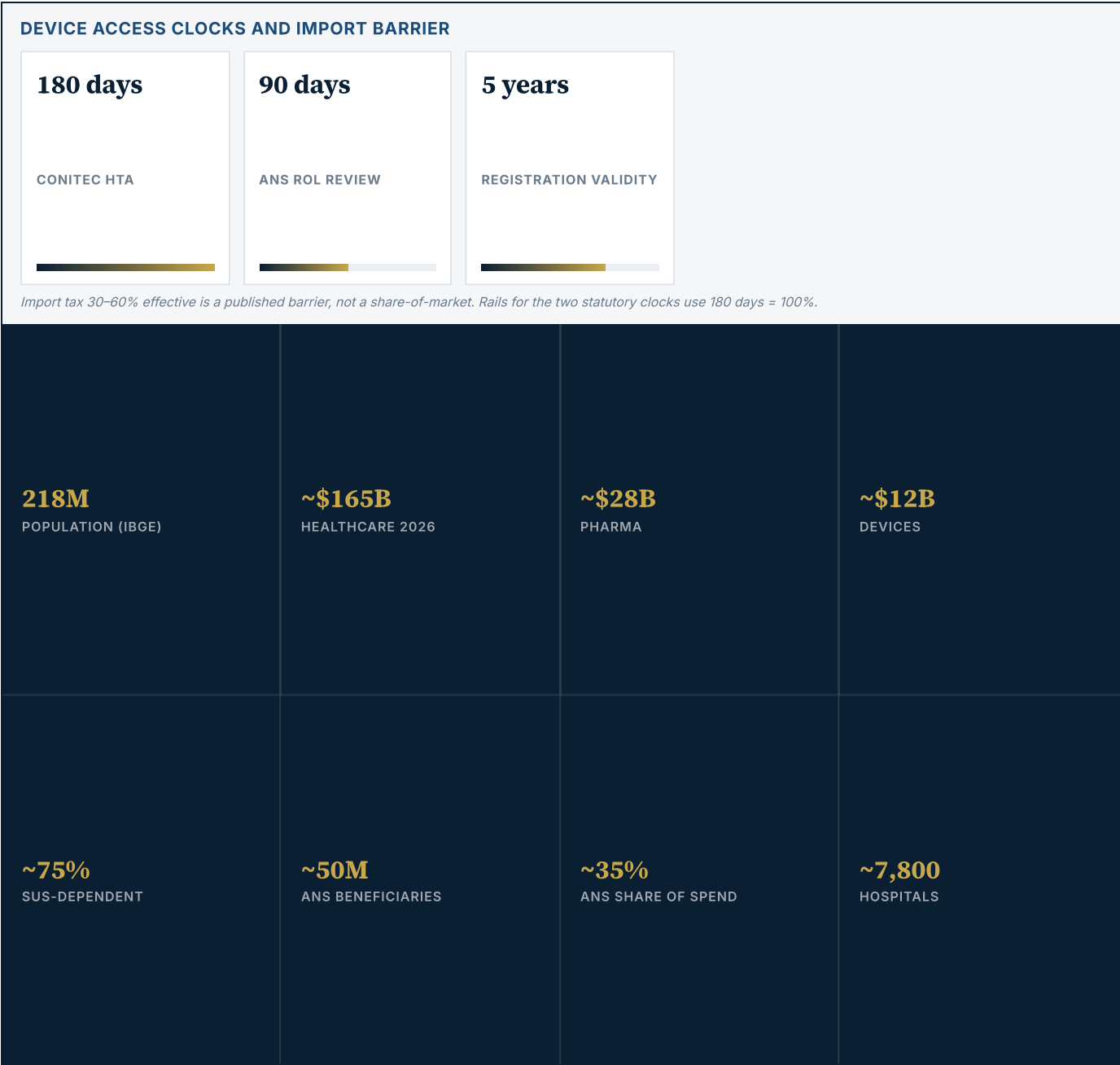
CONITEC — required for SUS-funded technologies; 180-day statutory HTA; Brazilian cost-effectiveness vs GDP/capita.
- 4

RENAME / RENASES listing — Ministry of Health; required for SUS coverage.
- 5

ANS Rol — 90-day statutory review for private-plan mandate (~50M insured).

Premium private hospitals operate at full international price points with limited price regulation versus the SUS channel.

Device KPI and import-tax note: BioNixus Brazil market intelligence. Access steps: same panel.



05 Therapy and epidemiology

The Brazil KPI panel does not publish therapy-area dollar sizes. Demand facts below are only those printed on the live report.

CONDITION	PUBLISHED STAT	SOURCE ON PAGE
Cancer	~625,000 new diagnoses/year; breast, prostate, colorectal, cervix most prevalent	INCA 2023
Cardiovascular disease	~400,000 deaths/year from CVD — #1 cause of mortality	SBC 2023
Diabetes	~16.8 million adults (~15.7% adult prevalence); FAQ: sixth-largest diabetic population globally	IDF Diabetes Atlas 2023

FAQ THERAPY RANKING (COMBINED PUBLIC + PRIVATE VALUE)



Bars are a rank illustration of the FAQ order — not published share-of-spend percentages. Do not treat widths as market share.

ANS expanded the Rol de Procedimentos in 2022 to include additional oncology therapies, biologics and gene therapies — a private-channel growth fact from the live FAQ.

EPIDEMIOLOGY PUBLISHED ON THE LIVE PAGE

625k

New cancers / yr

400k

CVD deaths / yr

16.8M

Adults with diabetes

Cancer and CVD bars share a count scale (625k = 100%). Diabetes is a prevalence headcount and is labelled only — not on the same unit scale.

218M POPULATION (IBGE)	~\$165B HEALTHCARE 2026	~\$28B PHARMA	~\$12B DEVICES
~75% SUS-DEPENDENT	~50M ANS BENEFICIARIES	~35% ANS SHARE OF SPEND	~7,800 HOSPITALS

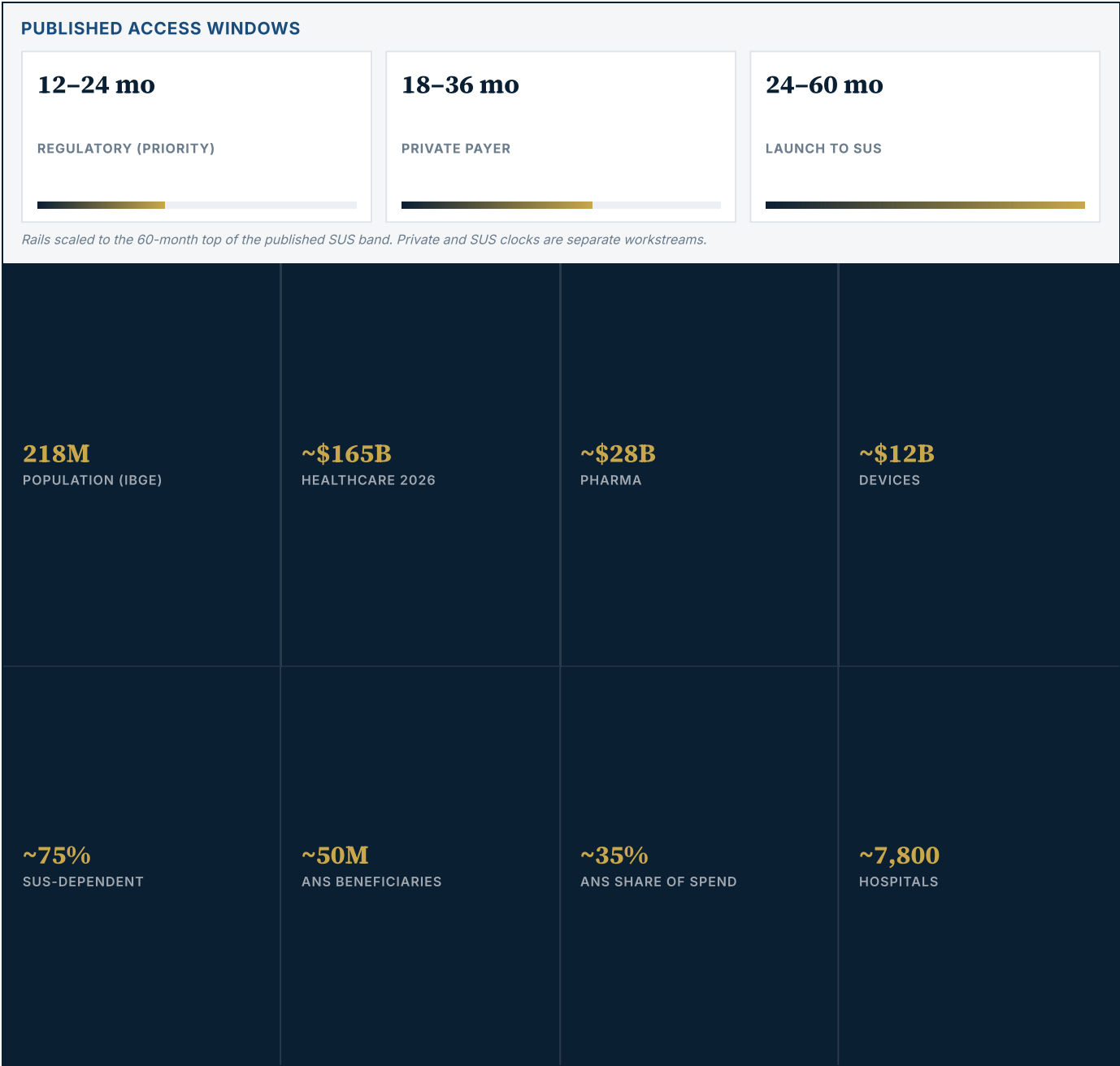
ANVISA → CONITEC → ANS

12–24 mo REGULATORY (PRIORITY)	6–12 mo PAYER LISTING	24–60 mo LAUNCH TO SUS ACCESS
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Private-payer clock on the KPI panel: **18–36 months**. FAQ CONITEC evaluation uses cost/QALY or cost/LYG plus budget impact from the SUS perspective. PCDT protocols set eligibility once listed.

- 1
- ANVISA pre-submission — reunião de esclarecimento; 30–60 days; pathway (priority / regular / abridged).
- 2
- ANVISA application — GAFAR/GGMED; eCTD innovative; CTD biologics; Module 1 Portuguese.
- 3
- ANVISA review — priority 120 days; regular 365 days (KPI). FAQ complexity band 12–36 months.
- 4
- Marketing approval — 5-year renewable; CNPJ required.
- 5
- CONITEC HTA — 180 days statutory; Brazilian CE evidence.
- 6
- RENAME / RENASES — Ministry of Health listing for SUS coverage.
- 7
- ANS incorporation — 90-day statutory; Rol de Procedimentos e Eventos em Saúde.

accessTimeline and registrationSteps: BioNixus Brazil market intelligence. FAQ supplements CONITEC / PCDT language.



07 Hospitals and dual-channel accounts

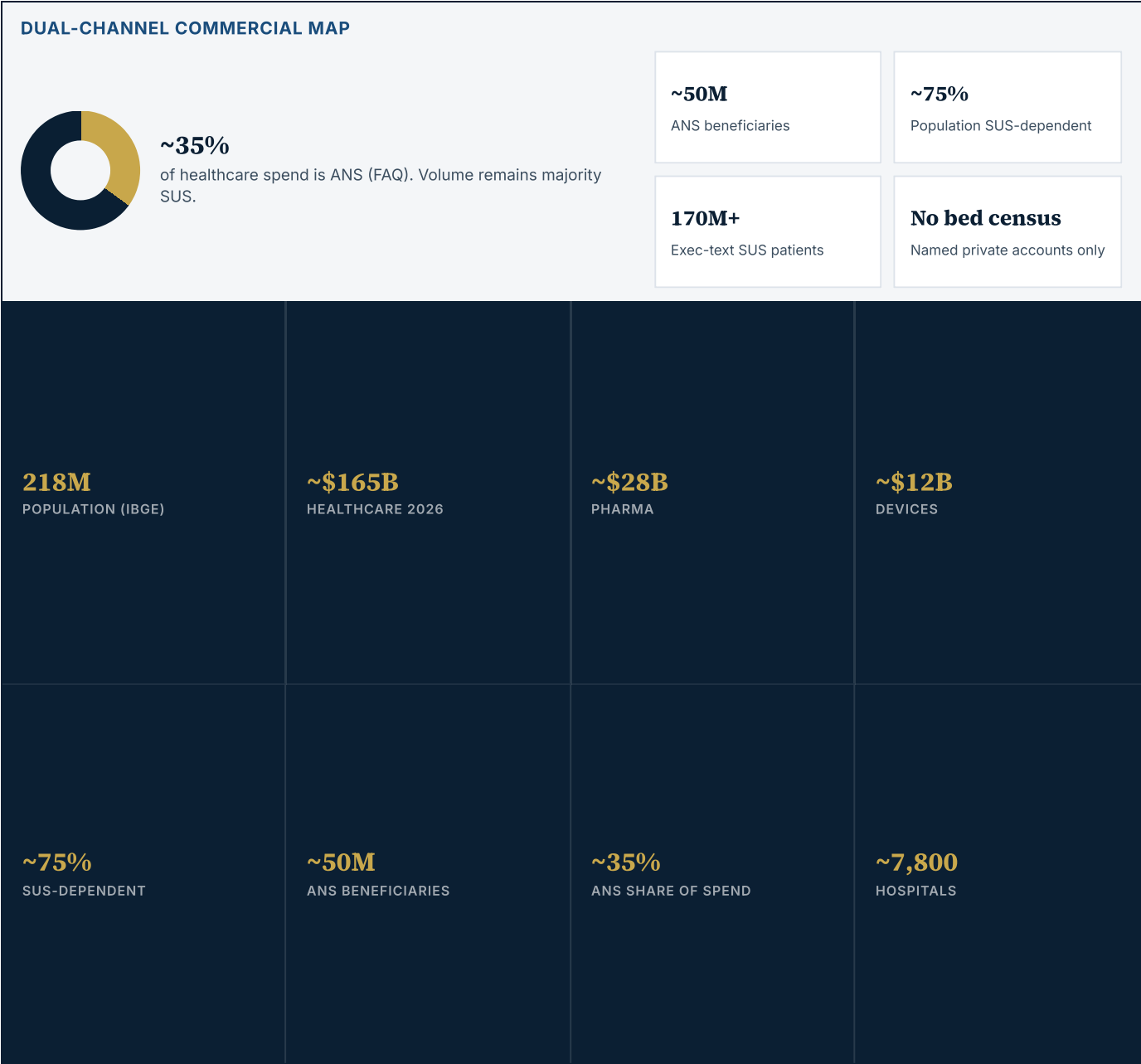
The Brazil KPI panel does not publish a numbered hospital list. Named accounts below appear on the live FAQ as the commercially relevant private set — bed counts are not invented.

- Hospital Albert Einstein** — premium private; international price points.
- Hospital Sírio-Libanês** — premium private; branded pharmaceuticals and novel devices.
- Hospital BP** — premium private supplementary-sector account.
- Hospital Mater Dei** — premium private supplementary-sector account.
- INCA-affiliated cancer centres** — public oncology protocols and RENAME-linked SUS volume.

HOW THE TWO CHANNELS DIFFER

	SUS (PUBLIC)	ANS (PRIVATE)
Population	Constitutional coverage; KPI ~75% dependent; exec text 170+ million patients	~50 million beneficiaries
Spend share	Majority of volume	~35% of total healthcare spend (FAQ)
Access gate	CONITEC + RENAME/RENASES + PCDT	ANS RoI (expanded 2022)
Price	CMED + public procurement	Closer to international list; limited vs SUS

Hospital names and ANS/SUS facts: Brazil FAQ and executive summary. Do not add unsourced bed counts.



08 Methodology and sources

Figures are taken from **/brazil-healthcare-market-report** (published 27 May 2026; modified **22 August 2026**) and the Brazil market-intelligence KPI panel. Sample edition: **September 2026**.

RECONCILIATION RULES

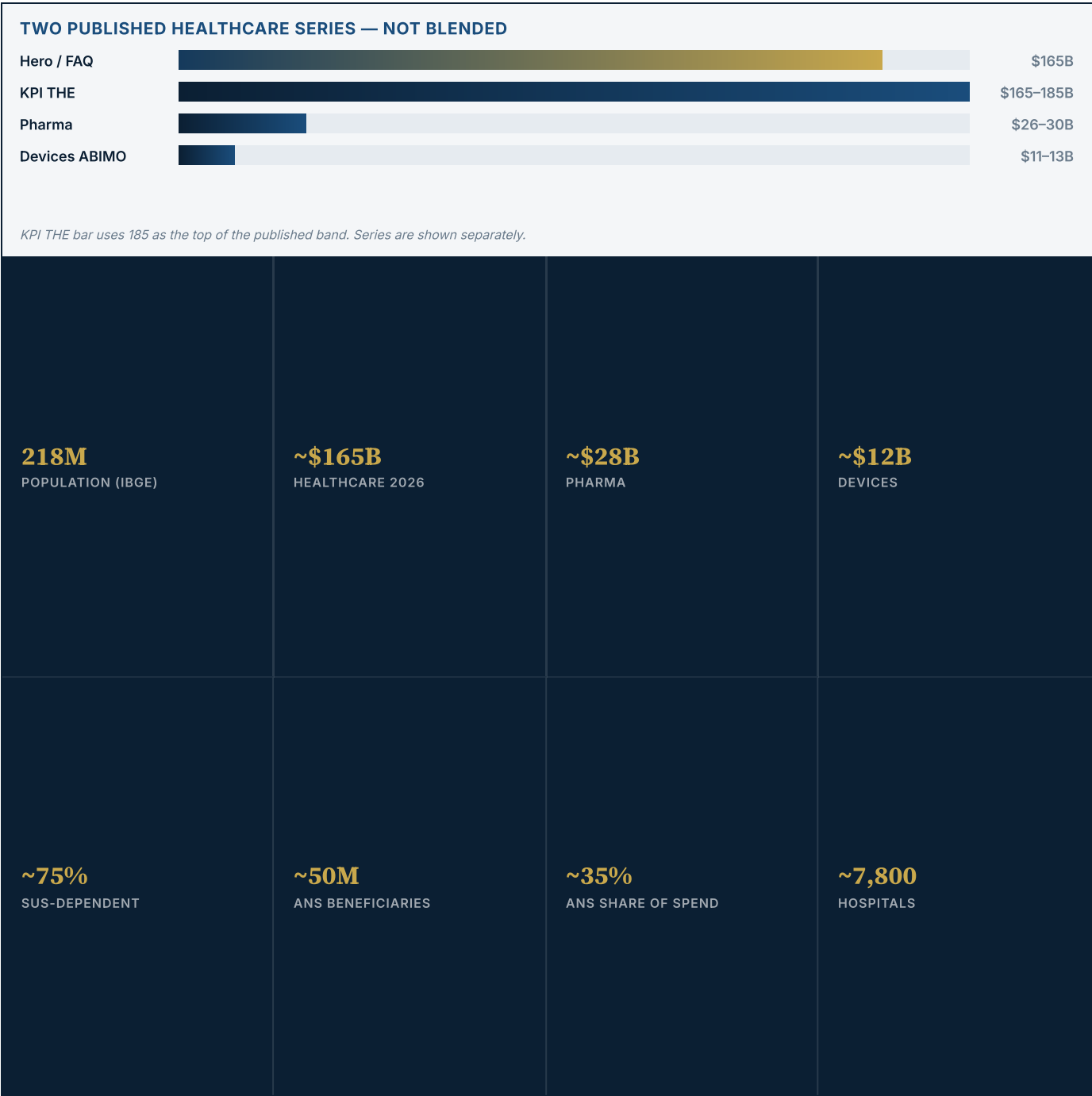
Headline chips: healthcare **~\$165B**, pharma **~\$28B**, devices **~\$12B**. FAQ healthcare band USD 155–175B. KPI THE USD 165–185B is shown as a second series. Pharma USD 26–30B is consistent across FAQ and KPI. Devices use ABIMO USD 11–13B behind the **~\$12B** chip. Population: KPI 218 million (IBGE); FAQ 215+ million citizens — both printed, not averaged.

CITED SECONDARY SOURCES (AS NAMED ON THE PAGE)

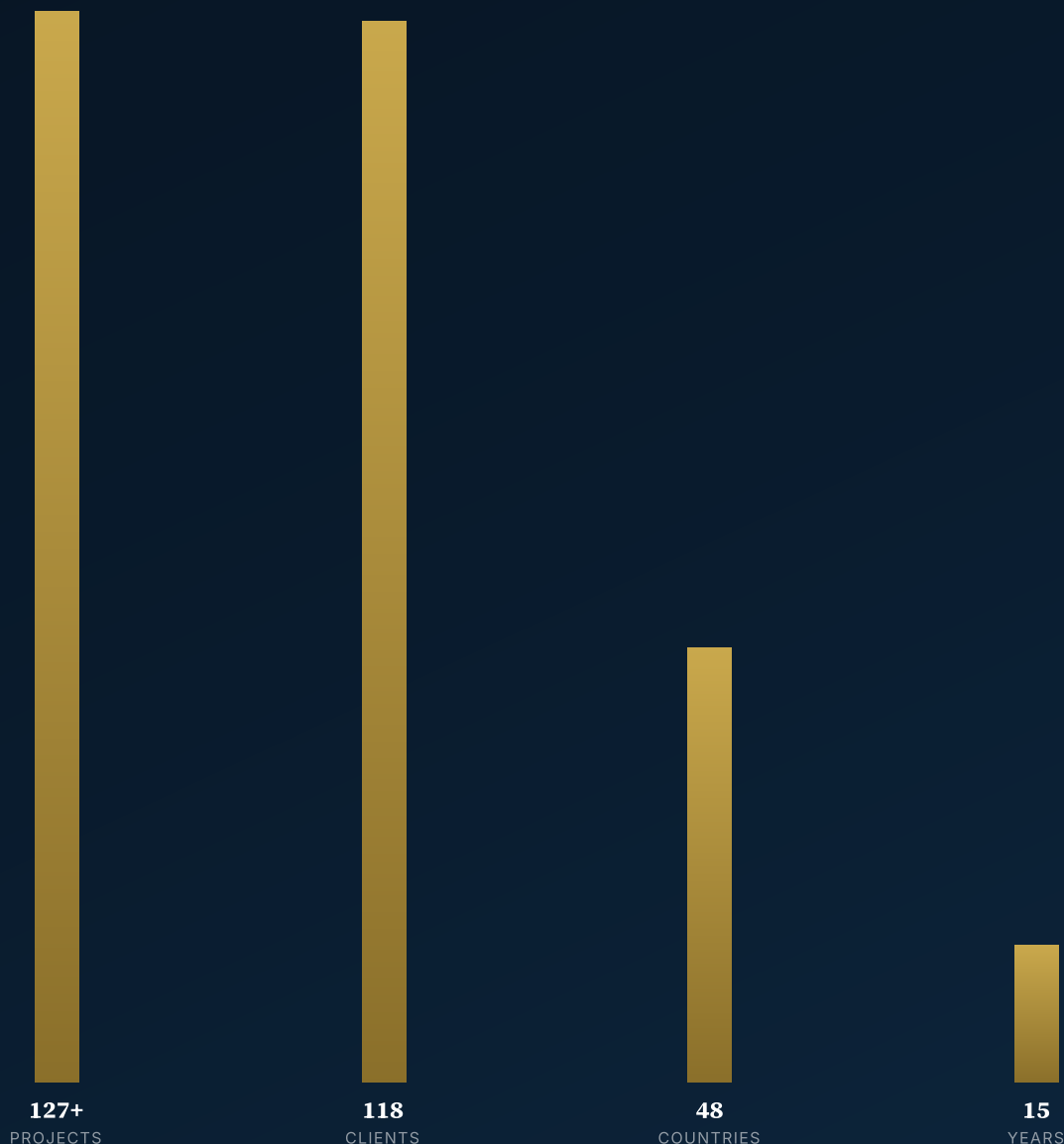
IBGE — population. IMF 2025 — GDP per capita. INTERFARMA — pharma KPI note. ABIMO — devices. INCA 2023, SBC 2023, IDF Diabetes Atlas 2023 — epidemiology. ANVISA, CONITEC, ANS, CMED — institutional facts from the same page.

Gated excerpt only. SUS tender calendars, ANS plan-level coverage and hospital-level consumption remain in commissioned work. Nothing herein is legal, medical, pricing or investment advice.

bionixus.com/brazil-healthcare-market-report



FIRM SNAPSHOT



NEXT STEP

Primary research, not a spreadsheet export.

BioNixus is a healthcare and pharmaceutical market research firm founded in 2012 — USA headquarters, London, Cairo and a Brazil / LatAm presence. We run physician, payer and patient primary research plus market access and HEOR across 48 countries.

Minimum engagement USD 20,000. Typical proposal: 48 hours.

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